



*Providing Creative Solutions to Common Problems*

POST-ACUTE & SKILLED NURSING RECOVERY · CAPABILITIES OVERVIEW

# Debt Collection Agency for Businesses

Third-party recovery of aged resident balances and stalled payer receivables for Midwest City Post Acute & Rehab — on pure contingency

Prepared for **Midwest City Post Acute & Rehab** · August 2026

## Third-party recovery for the balances your business office has already worked

Prepared for Midwest City Post Acute & Rehab — a skilled nursing and post-acute rehabilitation facility on National Avenue in Midwest City, taking admissions from SSM Health St. Anthony Midwest and hospitals across the Oklahoma City metro for orthopedic, stroke, cardiac and complex medical recovery. A post-acute census produces a receivable no other setting does: Medicare Part A coinsurance that starts on day 21, private-pay and patient-liability balances during Medicaid pending, room-and-board shortfalls after a payer source changes mid-stay, and balances left behind when a resident discharges home or transfers before the account is settled.

**Care already delivered is revenue already earned.** In post-acute the balance is rarely disputed — it is simply left behind. A short-stay resident discharges home on day 30 owing Part A coinsurance. A Medicaid application is denied or takes eight months and the private-pay liability accrues the whole time. A responsible party signs the admission agreement and stops answering after discharge. Your business office works these for as long as it can, and then the account sits.

Southwest Recovery Services is the third-party layer that starts there. We work healthcare receivables on **pure contingency since 2004** — under a Business Associate Agreement, with HIPAA and Regulation F training on every account. No retainer, no minimum, no recovery, no fee.

## Two decades of healthcare recovery — nationwide

A track record built on resident, patient and payer balances, worked under a documented, compliant process — capacity your business office can add without hiring for it.

|                                                        |                                                       |                                        |                                             |
|--------------------------------------------------------|-------------------------------------------------------|----------------------------------------|---------------------------------------------|
| <b>2004</b><br>Recovering commercial receivables since | <b>\$975M+</b><br>Receivables placed (all industries) | <b>\$76M+</b><br>Recovered for clients | <b>800K+</b><br>Accounts worked             |
| <b>52%</b><br>of placed accounts see a recovery        | <b>50</b><br>states — nationwide recovery reach       | <b>4.9★</b><br>Google client rating    | <b>\$0</b><br>upfront — no recovery, no fee |

Commercial & B2B collections since 2004

Nationwide — all 50 states

12 offices across 7 states

Skip tracing on businesses & principals

Contingency — no recovery, no fee

### RECOGNIZED & FEATURED BY

USA TODAY

FINANCIAL SERVICES  
Review

FSR • TOP 10 LIST  
2025 Recognition

INTERNATIONAL  
Business Times

CFO Tech  
Outlook

insideARM

### ACCREDITED & RECOGNIZED



 ★ 4.9 · 38 Google reviews

Memberships: **ACA International** · TAA · AAGD · NARPM

## What a post-acute business office asks us — answered directly

The questions that matter when the receivable is resident liability, Medicaid pending and Part A coinsurance across a rotating short-stay census.

| YOUR QUESTION                                                    | SOUTHWEST RECOVERY SERVICES                                                                                                                                                                                                                                                                                                                                                                                                                  |
|------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>What does third-party collections cost?</b>                   | Pure contingency — \$0 upfront, no retainer, no monthly minimum. You pay a pre-agreed percentage only of what we actually recover. The rate scales with the age, balance size and volume of what you place; we quote it once we see a representative sample of your aging.                                                                                                                                                                   |
| <b>Which balances should we place?</b>                           | The ones your business office has already worked and cannot move: Part A coinsurance after a short stay, private-pay and resident-liability balances from a Medicaid pending period that resolved unfavorably, room-and-board shortfalls after a payer change mid-stay, and any balance left behind at discharge or transfer. Ninety days past the last meaningful contact is the practical trigger.                                         |
| <b>How do you handle the resident and the responsible party?</b> | Carefully, and in that order. We work the responsible party named on the admission agreement wherever one exists, and we contact residents and families with the tone this setting requires — firm and professional, never aggressive. These are people whose family member received care from you, and often still lives in your building. That is not a place for pressure tactics, and our process is documented so it never becomes one. |
| <b>What about HIPAA?</b>                                         | Every healthcare client is worked under a Business Associate Agreement. Staff carry ongoing HIPAA and Regulation F training and we operate on the minimum-necessary standard — we work a balance without needing a clinical record. Admission agreements, itemized statements and remittance advice are all we require.                                                                                                                      |
| <b>What if a payer, not a person, owes the money?</b>            | Those are worked differently and lawfully. Managed-care and Medicare Advantage balances that stalled in follow-up, denied days, and unpaid coinsurance from a secondary are pursued as commercial receivables against the plan — a separate track from resident-liability accounts, and one where persistent, documented follow-up is usually what closes it.                                                                                |
| <b>How does placement work?</b>                                  | Upload an aging export from PointClickCare or whatever your business office runs through our secure portal — we map your columns, so nothing gets re-keyed. Outreach begins within 24–48 hours of a documented placement, and you can pull reports and log direct payments yourself at any time.                                                                                                                                             |

## Business-office-ready intake and full visibility, on demand

Every placement is managed through our secure portal — built to take an aging export from your practice-management or long-term-care system and give your business office live, self-service access throughout recovery.

### AGING-EXPORT-READY PLACEMENT

Upload an export from PointClickCare or your billing system — we map your columns, no re-keying.

### REAL-TIME REPORTING

Run and export account and recovery reports on demand — a live view of your aged AR instead of a monthly spreadsheet.

### SECURE DOCUMENT EXCHANGE

Send admission agreements, itemized statements, responsible-party paperwork and remittance advice safely inside the portal.

### DIRECT-PAYMENT REPORTING

Log payments a resident or family makes straight to the facility, so balances stay accurate and no one is contacted twice.

► [Portal Video Tour](#)

## SWRS Healthcare Recovery Case Studies

Actual placed-versus-recovered results from Southwest Recovery's healthcare book, anonymized by sector.

### SKILLED NURSING & REHABILITATION

Post-acute care center

**\$58K** recovered

of ~\$88K placed

65% recovered

### SENIOR LIVING COMMUNITY

Assisted & independent  
living

**\$25K** recovered

of ~\$30K placed

86% recovered

### NURSING & REHABILITATION CENTER

Single-site SNF

**\$23K** recovered

of ~\$41K placed

55% recovered

### MULTI-SITE NURSING GROUP

Regional SNF operator

**\$79K** recovered

of ~\$195K placed

41% recovered

### THERAPY SERVICES

PROVIDER

Contract rehab therapy

**\$43K** recovered

of ~\$43K placed

100% recovered

### LONG-TERM CARE OPERATOR

Nursing & rehab center

**\$47K** recovered

of ~\$156K placed

30% recovered

Anonymized by sector from Southwest Recovery's book; individual client names withheld. Results vary by account age, balance, documentation quality and industry. We do not publish blended portfolio averages.

**WHY EARLY PLACEMENT  
MATTERS**

## **A resident balance is most collectible while the stay is recent**

Post-acute balances decay faster than most because the person owing the money has already gone home. Every month after discharge, contact information goes stale, the responsible party disengages, and the balance drifts toward write-off.

### **THE AGE CURVE**

Balances placed within 90–120 days of becoming resident responsibility recover materially better than balances that sit a year past discharge.

### **THE STANDING RULE**

A fixed placement policy beats case-by-case decisions, and on contingency there is no cost to placing.

*Recovery figures vary by portfolio and payer mix; SWRS gives an honest, age-based read on a representative sample before any accounts are worked.*

## We speak your ledger

A short-stay census running Medicare Part A, Medicaid pending, managed care and private pay produces a specific recovery profile. Our playbook is built for it.



### Part A coinsurance after discharge

Day-21-forward coinsurance on a short stay, owed by a resident who has already gone home — worked from your admission agreement and itemized statement.



### Medicaid pending that resolved badly

Private-pay and resident-liability balances that accrued through a long pending period or a denial — the largest single write-off category in most facilities.



### Responsible parties who stop answering

Locating the signer on the admission agreement after discharge, a move, or a change in family circumstances — so the balance stays attached to the obligor.



### Payer balances, worked separately

Managed-care and Medicare Advantage receivables stalled in follow-up, pursued as commercial balances against the plan rather than the family.



## Recovery run on an AI-driven platform — Aktos

Every account SWRS works runs on Aktos, a next-generation, AI-driven collections platform — with compliance built into the workflow, not bolted on afterward. It's how a boutique, people-first agency delivers enterprise-grade speed, consistency, and auditability.



### AI workflow automation

Repetitive steps are automated and every account is scored and routed by predicted payment likelihood — so the right strategy reaches the right account first, without manual queue-picking.



### Omnichannel outreach

Coordinated email, SMS, and voice, timed to when a debtor is most likely to engage — replacing high-friction call blasts with respectful, data-driven contact.



### Compliance built in

FDCPA, Reg F, and TCPA guardrails are enforced inside the automation, with phone-number and consent checks before outreach — documented and defensible on every account.



### Real-time dashboards

You see status, segmentation, and recovery performance on your portfolio in real time — full transparency, not a monthly PDF.



### **API-first intake**

Secure SFTP/API placement means accounts start working without data lag — roughly 50% faster acknowledgment and validation of new placements.



### **Data-driven scoring**

Dynamic scoring and segmentation predict payment likelihood and concentrate effort where recovery is most achievable — earlier intervention on aged balances and fewer disputes.

The result is the combination clients rarely get from a legacy agency: faster cash and cleaner compliance at the same time. Automation accelerates recovery and standardizes documentation, while empathy-first, data-driven contact protects your brand and keeps disputes and complaints low.

## Built by a collector with 30 years on the phones — not a CEO with zero collection experience



### Steven Dietz

Founder & Chief Executive Officer

Featured in USA Today, International Business Times & Financial Services Review — Top AR Management Service, 2025 · ACA International member.

Steven Dietz didn't come to collections from the corner office — he came from the phones. He started as a collector and credit analyst before he was 20, was tapped by Aaron's to train collectors across 100+ locations, and has spent nearly 30 years in receivables. Along the way he became convinced the industry's bad reputation was a choice, not a requirement: businesses get paid faster when recovery is built on listening and respect rather than aggressive calls and empty threats.

He founded Southwest Recovery Services in 2004 on that principle — a national, family-owned receivables partner that treats every debtor as a person and every client relationship as worth protecting. Two decades later he still coaches the teams and works accounts personally — an operator who has actually done the job, not a manager who has only overseen it.

Regardless of the means of recovery, our focus is always on preserving goodwill and representing the client professionally.

— Steven Dietz, Founder & Chief Executive Officer, Southwest Recovery Services

## Take it with you

Download this overview and the supporting white papers to share with your team.



### **This capabilities overview**

PDF · prepared for Midwest City Post Acute & Rehab

[Download PDF ↓](#)



### **Recovering Patient A/R**

Early-out & bad debt · PDF

[Download PDF ↓](#)



### **Medical Debt Collection Compliance**

HIPAA / FDCPA / state rules · PDF

[Download PDF ↓](#)



### **The Aktos Advantage**

Our AI-driven collections platform · PDF

[Download PDF ↓](#)



### **The Experian Intelligence Advantage**

Data & skip-tracing stack · PDF

[Download PDF ↓](#)



### **The Recovery Waterfall**

Full recovery lifecycle · PDF

[Download PDF ↓](#)

## Let's turn your aged resident balances back into revenue.

A 30-minute call to walk a representative sample of Midwest City Post Acute & Rehab's aged resident and payer balances, confirm what is lawfully collectible across your payer mix, and put a contingency rate against it.

[Book a 30-minute discovery call →](#)

### Raymond Davidson

Director of Client Services

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12 offices across 7 states · nationwide recovery. Private & confidential —  
prepared for Midwest City Post Acute & Rehab. FDCPA / CFPB / FCRA compliant ·  
ACA International member.